1mg Labs

LFS Healthcare Pvt Ltd (1mglabs)
LABORATORY : 82/A, Ground Floor, Udyog Vihar,
Phase 4, Behind Airtel Building, Gurgaon - 122 015
Website : www.1mglabs.com
Mail Id : care@1mglabs.com
Phone No. : 0124-4006767
CIN : U85100HR2015PTC057368

REGISTERED OFFICE :
5th Floor, Tower- B,
The Presidency Building, 46/4,
Mehrauli Gurgaon Road, Sector 14
Gurgaon - 122 001

ITDOSE INFOSYSTEMS PVT. LTD.

Name:	**NIPPUN RANA**	Order ID:	1573189
Age/Gender:	29 Y/Male	Registration Date:	16/Feb/2020 12:00PM
Patient ID:	012002160211	Collection Date:	16/Feb/2020 09:04AM
Barcode ID:	A1378269	Sample Receive Date:	16/Feb/2020 02:25PM
Referred By:	Self	Report Status:	Final
SampleType:	WB EDTA	Report Date:	16/Feb/2020 04:04PM

NABL CERTIFICATE : MC-3226

HAEMATOLOGY

Max Bupa HBV4 C Package

Test Name	Result	Unit	Bio Ref.Interval	Method
Complete Blood Count				
Hemoglobin	14.0	g/dL	13.0 - 17.0	Cyanide-free SLS-Hemoglobin
RBC	4.67	mili/cu.mm	4.5 - 5.5	DC Impedence Method
PCV	42.6	%	40 - 54	Calculated
MCV	91.2	fL	83 - 101	Calculated
MCH	30.0	pg	27 - 32	Calculated
MCHC	32.90	g/dL	32 - 35	Calculated
RDW-SD	41.70	fL	39 - 46	Electronic Impendance
RDW-CV	12.20	%	11.6 - 14.0	Calculated
Total Leucocyte Count	7.0	10^3/µl	4 - 10	Flowcytometery/Microscopic
Differential Leucocyte Count				
Neutrophils	43	%	40 - 80	Flowcytometery/Microscopic
Lymphocytes	**45**	%	20 - 40	Flowcytometery/Microscopic
Monocytes	8	%	2 - 10	Flowcytometery/Microscopic
Eosinophils	4	%	1 - 6	Flowcytometery/Microscopic
Basophils	0	%	0 - 2	Flowcytometery/Microscopic
Immature Granulocyte Count	0	%		Flowcytometery/Microscopic
Absolute Leucocyte Count				
Absolute Neutrophil Count	3.01	10^3/µl	2.0 - 7.0	Calculated
Absolute Lymphocyte Count	**3.15**	10^3/µl	1.0 - 3.0	Calculated
Absolute Monocyte Count	0.56	10^3/µl	0.2 - 1.0	Calculated
Absolute Eosinophil Count	0.28	10^3/µl	0.02 - 0.50	Calculated
Absolute Basophil Count	0	10^3/µl	0.0 - 0.10	Calculated
Absolute Immature Granulocyte Count	0	10^3/µl		Calculated
Platelet Count	297	10^3/µl	150 - 410	Microscopic
MPV	10.3	fL		Calculated
PDW	12.70	fL		Calculated

Kindly correlate clinically

Dr. Shweta Agarwal
MD (Pathology)

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Barcode ID:	A1378270	Sample Receive Date:	16/Feb/2020 02:25PM
Referred By:	Self	Report Status:	Final
SampleType:	WB EDTA	Report Date:	16/Feb/2020 04:36PM

NABL CERTIFICATE : MC-3226

HAEMATOLOGY

Max Bupa HBV4 C Package

Test Name	Result	Unit	Bio Ref.Interval	Method
Erythrocyte Sedimentation Rate	**13**	mm/hour	<=10	Modified Westergren at 18°C

ESR measurement by the conventional method of Westergren, suggested by the International Committee for Standardization in Hematology as the standard procedure, provides an index of progress of the disease and is widely used in clinical practice as an indicator of inflammation, infection, trauma, or malignant diseases. ESR is specifically indicated for certain conditions: to monitor rheumatoid arthritis, tuberculosis, and systemic lupus erythematosis; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica. An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection, and malignancy. Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors, In polycythemia, the mass of the red blood cells increases which cause the decrease of ESR. In Anemia, the mass of the red cells decreases which cause the increase of ESR. In other words, Erythrocyte Sedimentation Rate is indirectly proportional to ratio between mass of the red cells and plasma.The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, a1-antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.

Technical Issues: Elevation in room temperature also affects the Erythrocyte Sedimentation Rate. Moving tube in sloping position, length and calibre of the tube also affect Erythrocyte Sedimentation Rate.

Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

Kindly correlate clinically

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Patient ID:	012002160211	Collection Date:	16/Feb/2020 09:04AM
Barcode ID:	A1378268	Sample Receive Date:	16/Feb/2020 02:29PM
Referred By:	Self	Report Status:	Final
SampleType:	Serum	Report Date:	16/Feb/2020 05:34PM

NABL CERTIFICATE : MC-3226

BIOCHEMISTRY

Max Bupa HBV4 C Package

Test Name	Result	Unit	Bio Ref.Interval	Method
Cholesterol	**218**	mg/dl	Desirable <200, Borderline High 200 - 239, High >=240	CHOD-POD

Kindly correlate clinically

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Barcode ID:	A1378269	Sample Receive Date:	16/Feb/2020 02:25PM
Referred By:	Self	Report Status:	Final
SampleType:	WB EDTA	Report Date:	16/Feb/2020 04:57PM

BIOCHEMISTRY

Max Bupa HBV4 C Package

Test Name	Result	Unit	Bio Ref.Interval	Method
Glycosylated Hemoglobin (HbA1c)	4.6	%	4 - 6	HPLC - Cation Exchange

As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>=6.5
Therapeutic goals for glycemic control	Age> 19 years Goal of therapy: <7.0 Action suggested: >8.0 Age <19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Average Blood Glucose	**85.32**	mg/dl	Mentioned below	Calculated

Range for Average Blood Glucose:

Excellent Control	90 – 120 mg/dl
Good Control	121 – 150 mg/dl
Average	151 – 180 mg/dl
Action Suggested	181 – 210 mg/dl
Panic Value	>=211 mg/dl

Average Blood Glucose is a calculated parameter from HbA1c value and it indicates average glucose over the past 3 months.

Kindly correlate clinically

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CERTIFIED ISO 9001:2015 COMPANY

ITDOSE INFOSYSTEMS PVT. LTD.

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Patient ID:	012002160211	Collection Date:	16/Feb/2020 09:04AM
Barcode ID:	A1378268	Sample Receive Date:	16/Feb/2020 02:29PM
Referred By:	Self	Report Status:	Final
SampleType:	Serum	Report Date:	16/Feb/2020 06:19PM

BIOCHEMISTRY

Max Bupa HBV4 C Package

Test Name	Result	Unit	Bio Ref.Interval	Method
LDL Cholesterol	**166**	mg/dl	Desirable: <100 Above desirable: 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >=190	Calculated

Kindly correlate clinically

Dr. Shweta Agarwal
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SampleType:	Serum	Report Date:	16/Feb/2020 05:34PM

NABL CERTIFICATE : MC-3226

BIOCHEMISTRY

Max Bupa HBV4 C Package

Test Name	Result	Unit	Bio Ref.Interval	Method
Kidney Function Test.				
Urea	26	mg/dl	14.98-38.52	Calculated
Blood Urea Nitrogen	12.00	mg/dL	7.0- 18.0	UREASE-GLDH
Creatinine	0.77	mg/dl	0.70 - 1.30	Alkaline picrate-kinetic
Uric Acid	**10.3**	mg/dl	3.5 - 7.2	Uricase
Sodium	**135**	mmol/l	136 - 145	Indirect ISE
Potassium	4.6	mmol/L	3.5 - 5.1	Indirect ISE
Chloride	99	mmol/L	98 - 107	Indirect ISE
BUN/Creatinine Ratio	15.6	Ratio		Calculated
Urea/Creatinine Ratio	33.35	Ratio		Calculated

Renal function tests help to screen the individual for renal disease and to determine the extent or progression of renal disease. These tests also aid in determining drug dosage for drugs excreted through the kidneys. The clinical syndrome resulting from decreased renal function and azotemia is called uremia
Renal azotemia: glomerular nephritis and chronic pyelonephritis.Prerenal azotemia: severe dehydration, hemorrhagic shock and excessive protein intake. Post renal azotemia: urethral stones or tumors and prostatic obstructions
Measurement of urea in dialysis fluids is widely used in assessing the adequacy of renal replacement therapy.
In these prerenal situations, the plasma creatinine concentration may be normal.In obstructive post renal conditions, both plasma creatinine and urea concentrations will be increased, although there is often a greater increase in plasma urea than creatinine because of the increased back diffusion.These considerations give rise to the principal clinical utility of plasma urea, which lies in its measurement in conjunction with that of plasma creatinine and subsequent calculation of the urea nitrogen/creatinine ratio. This ratio has been used as a crude discriminator between prerenal and postrenal azotemia. Significantly lower ratios usually denote (1) acute tubular necrosis, (2) low protein intake, (3) starvation, or (4) severe liver disease (decreased urea synthesis). So even though blood urea is not an excellent marker of renal dysfunction as it rises quite late in the dysfunction and its rise is also not exclusive to kidney dysfunction, but for practical purposes serum urea level is still one of the most ordered test and forms an important part of the kidney function test.
Long-term follow-up of asymptomatic hyperuricemic patients is undertaken because many are at risk for kidney disease that may develop as a result of hyperuricemia and hyperuricuria; few of these patients ever develop the clinical syndrome of gout.. It is also used in the diagnosis and monitoring of pregnancy-induced hypertension (pre-eclamptic toxemia). Concentrations in excess of 6.0 mg/dL at 32 weeks gestation have been noted to be associated with a high perinatal mortality rate.

HDL Cholesterol	**32**	mg/dl	40 - 60	Direct Measure- PEG

Kindly correlate clinically

Dr. Shweta Agarwal
MD (Pathology)

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Patient ID:	012002160211	NABL CERTIFICATE : MC-3226	Collection Date:	16/Feb/2020 09:04AM
Barcode ID:	a1378271		Sample Receive Date:	16/Feb/2020 03:26PM
Referred By:	Self		Report Status:	Final
SampleType:	Urine		Report Date:	16/Feb/2020 08:30PM

CLINICAL PATHOLOGY

Max Bupa HBV4 C Package

Test Name	Result	Unit	Bio Ref.Interval	Method
Urine Routine & Microscopy				
Colour	PALE YELLOW		Pale Yellow	Manual
Specific gravity	1.005		1.005 - 1.030	pKa change
Appearance	CLEAR		Clear	Manual
Ph	6.50		5.0 - 8.5	Double Indicator
Urobilinogen	NORMAL		Normal	Azo Dye
Leucocytes	NEGATIVE		Negative	Microscopy
Urine protein	NEGATIVE		Negative	Acid Base Indicator
Ketones	NEGATIVE		Negative	Acetoacetic Acid
Nitrite	NEGATIVE		Negative	Sulbhanilamide Diazo
Urine Glucose	NEGATIVE		Negative	Benedict test
Bile pigments	NEGATIVE		Negative	Fouchet Reagent
Blood	NEGATIVE		Negative	Microscopy
Bile salts	NEGATIVE		Negative	Hays sulphur
Epithelial cells	1-2		0 - 5/HPF	Microscopy
Urine RBC	NIL		Nil	Microscopy
Pus cells	1-2		0 - 5/HPF	Microscopy
Casts	NIL		Nil	Microscopy
Crystals	NIL		Nil	Microscopy
Budding yeast cells	NIL		Nil	Microscopy
Bacteria	NIL		Nil	Microscopy

*** End Of Report ***

Kindly correlate clinically

Dr. Rahul Goyal
MD (Pathology)

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Why Do Preventive Test?

Reality Check

60%
deaths are due to preventable diseases

50%
are at risk of heart disease

59
The mean age of heart failure patients in India

68%
Urban Indians do not practice preventive healthcare

20%
Population suffers from one of the preventable diseases

50 million
people in India suffer from diabetes, making it the Diabetes Capital of the World.

Average cost per case of hospitalisation in urban India is **₹26,455**

How much does it cost you to avoid getting sick?

₹ 1 spent on prevention saves **₹ 133** on absenteeism cost and **₹6.62** in healthcare costs.

Prevent illness instead of treating them!

Comprehensive Full Body Check-up

(105 tests)

Contains tests of Liver, Kidney, Heart, Vitamins, Diabetes, etc.

Women Wellness Package

(35 tests)

Contains tests of thyroid, hormones, Iron studies etc.

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